

UNI HIST Syrup

ANTI HISTAMINE (H₁-RECEPTOR)/ DECONGESTANT

Each 5 mL contains:
Brompheniramine maleate 4 mg
Phenylephrine hydrochloride 10 mg

♦ PRODUCT DESCRIPTION:

Clear, purple syrup with grape flavor

♦ MECHANISM OF ACTION:®

Pharmacology

Antihistaminic (H₁-receptor): Antihistamines used in the treatment of allergy act by competing with histamine for H₁-receptor sites on effector cells. They thereby prevent, but do not reverse, responses mediated by histamine alone. The anticholinergic actions of most antihistamines provide a drying effect on the nasal and oral mucosa.

Decongestant: Sympathomimetic amines act on alpha-adrenergic receptors in the mucosa of the respiratory tract to produce vasoconstriction, which temporarily reduces the swelling associated with inflammation of the mucous membranes lining the nasal passages.

Pharmacokinetics

Absorption: Phenylephrine has reduced bioavailability (about 38%) from gastrointestinal tract because of first pass metabolism by monoamine oxidase in the stomach and liver.

Biotransformation: Antihistamines - Hepatic (cytochrome P-450 system); some renal;

Sympathomimetic amines – Phenylephrine: Extensive in the intestinal wall and in the liver. Sulfate conjugates are formed largely in the intestinal wall. Also, undergoes oxidative deamination by monoamine oxidase.

Half-life: Brompheniramine - 25 hours; Phenylephrine - 2.1 to 3.4 hours.

Onset of action: Most first generation antihistamines: 15 to 60 minutes.

Time to peak concentration: Brompheniramine - 2 to 5 hours; Phenylephrine - 0.75 to 2 hours (to achieve peak levels ranging from 0.9 to 298 ng/mL, respectively).

Time to peak effect: Brompheniramine - 3 to 9 hours;

Elimination: Antihistamines - Renal. Most of the antihistamines studied are excreted as metabolites within 24 hours; Phenylephrine: 2.6% of the administered oral dose is excreted unchanged; 80 to 86% of unchanged Phenylephrine and metabolites are recovered in 48 hours after oral administration.

♦ INDICATION:

For the symptomatic relief of nasal congestion due to cough and cold and allergic conditions, mainly rhinitis.

♦ DOSAGE AND ADMINISTRATION:

Oral

Adults
 5 mL (1 teaspoonful) every 4 to 6 hours

Children

6 – 12 years: 2.5 mL (½ teaspoon) every 4 to 6 hours

♦ PRECAUTION:

UNI HIST Syrup may cause drowsiness and may increase the effects of alcohol. Drowsiness may continue the following day. Those affected should not drive or operate machinery; alcohol should be avoided. Use with caution in patients with renal or hepatic impairment and in patients with epilepsy. Phenylephrine has an excessive vasopressor response may cause a prolonged rise in blood pressure. It induces tachycardia or reflex bradycardia and should therefore be avoided in severe hyperthyroidism and used with caution in severe ischaemic heart disease. Patients with diabetes mellitus or prostatic hyperplasia should also avoid Phenylephrine.

Pregnancy and lactation

UNI HIST Syrup should be used in pregnancy only if the potential benefits to the patient are weighed against the possible risk to the fetus. It is not recommended for breastfeeding mothers unless the potential benefits to the patient are weighed against the possible risk to the infant.

♦ WARNING:®

- Not recommended for children below 2 years of age.

- Exercise caution and follow the instruction of doctor or pharmacist when giving to children between 2 to 6 years of age.

♦ SIDE EFFECT:

Central Nervous System (CNS)

CNS depressive effects include sedation and impaired performance (impaired driving performance, poor work performance, incoordination, reduced motor skills and impaired information processing). Performance may be impaired in the absence of sedation and may persist the morning after a night-time dose. CNS stimulatory effects may include anxiety, hallucinations, appetite stimulation, muscle dyskinesia and activation of epileptogenic foci.

Anticholinergic effects

Side effects associated with cholinergic blockage include dryness of the eyes, mouth and nose, blurred vision, urinary hesitancy and retention, constipation and tachycardia.

Others

Skin rashes and urinary retention in children and elderly are more likely to experience adverse effects than other age groups.

♦ CONTRAINDICATION:

A history of hypersensitivity to the substance or substances of similar chemical structure (or any of the other ingredients in the product).

♦ DRUG INTERACTION:

The following interactions have been noted: Central Nervous System (CNS) depressants (alcohol, sedatives, opioid analgesics, hypnotics); May cause an increase in sedation effects monoamine oxidase inhibitors (MAOIs) and tricyclic antidepressants (TCAs); May prolong and intensify the anticholinergic and CNS depressive effects other sympathomimetic agents, such as decongestants, appetite suppressants and amphetamine-like psychostimulants; May cause an increase in blood pressure and additive effects methyl dopa and β-blockers; May cause an increase in blood pressure.

♦ OVERDOSE AND TREATMENT:

Symptoms

Nervousness, tremor, insomnia, agitation, irritability, abdominal pain, severe headache, nausea and vomiting.

Treatment

Stomach should be emptied by aspiration and lavage. Diazepam can be given to control CNS stimulation and convulsions. Alpha-adrenergic blockers are useful to manage hypertension.

♦ STORAGE:

Store at temperature of not more than 30°C.

♦ DOSAGE FORM AND PACKAGING AVAILABLE:

Syrup, Bottle by 100 mL

♦ DATE OF REVISION:

September 27, 2023

Manufactured by:
UNISON LABORATORIES CO., LTD.
 39 Moo 4, Klong Udomchokjorn, Muang Chachoengsao,
 Chachoengsao 24000 Thailand

C-MY/ 270923-13 (AR)

(USP DI: Thomson Micromedex Series 2006® and Additional information from Medispec, NPCL/ BPFK®)

IUMY 0112

13 cm.

19 cm.

UNI HIST SYRUP_Insert_FrontBack_IUMY 0112
 By_NUY 28/09/2023

Product	Code No.	Packaging Type	Thickness
UNI HIST SYRUP	IUMY 0112	Wood Free Paper (mammalian)	0.08 (0.0) 6 09
PM Specification	Designed by: (PMA ASD PQCM PQCA PQCM PQCA)	Checked by: (PMA ASD PQCM PQCA PQCM PQCA)	Approved by: (PMA ASD PQCM PQCA PQCM PQCA)
	Designed by: (PMA ASD PQCM PQCA PQCM PQCA)	Checked by: (PMA ASD PQCM PQCA PQCM PQCA)	Approved by: (PMA ASD PQCM PQCA PQCM PQCA)
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